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Correct Answer

Next Question

Practice Exam - 1

Question 51 of 117



A 33-year old female is admitted with a 4-week onset of progressive dyspnea on exertion and leg edema. Her medical history is otherwise notable for several years of a nonproductive cough, a history of obesity and gestational diabetes. On examination, blood pressure is 144/80 mm Hg, heart rate is 65 beats per minute.

Jugular venous pressure is 15 mm H₂O. Cardiac examination shows regular rate and rhythm without murmurs. Lower extremities have 1+ edema bilaterally.

Electrocardiogram demonstrates sinus rhythm with low-normal voltage and a right bundle branch block. On transthoracic echocardiography she has normal right and left ventricular systolic function, with mitral E to A velocity reversal. The patient is effectively diuresed during hospitalization, with symptomatic improvement, and initiated on an oral diuretic. During her hospitalization she is noted to have several episodes of second degree Mobitz type II heart block.

Which of the following additional tests should be considered in this patient with new heart failure and preserved ejection fraction?

✓ **A.** Cardiac fluorodeoxyglucose-positron emission tomography imaging

✗ **B.** Technetium pyrophosphate scan

C. Coronary angiography

D. Genetic counseling and testing

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize the importance of identifying the underlying etiology in patients with heart failure with preserved ejection fraction.

Answer: A. Cardiac fluorodeoxyglucose-positron emission tomography imaging (FDG-PET). This patient presents with new heart failure with preserved ejection fraction. Risk factors for heart failure with preserved ejection fraction in the setting of cardiometabolic

syndrome include her history of diabetes and obesity, and she is also noted to be hypertensive during this admission. However, there are several additional key historical points to consider and prompt further evaluation for specific underlying etiologies for heart failure in the setting of preserved ejection fraction. Key history includes the patient's young age, conduction system disease and low voltage on ECG which should prompt consideration of infiltrative cardiomyopathies. Given her young age and chronic cough, sarcoidosis is the most likely possibility. Cardiac sarcoidosis specifically can present with ventricular arrhythmias, conduction disease and/or heart failure either with systolic ventricular dysfunction or as a restrictive phenotype with preserved ejection fraction. Diagnosis of cardiac sarcoidosis is made through a combination of histopathologic findings and advanced cardiac imaging including cardiac FDG-PET and magnetic resonance (CMR) imaging, both which would be reasonable in this case. Diagnosis carries treatment implications, namely in the form of immunosuppression.

Answer B: Though the low voltage and conduction disease should also raise suspicion for amyloidosis, the patient's younger age and other presenting features such as chronic cough make sarcoidosis more likely than transthyretin amyloidosis for which technetium pyrophosphate scan may be useful.

Answer D: Genetic counseling would be indicated if family history and/or further cardiac imaging reveal concern for an inherited infiltrative cardiomyopathy.

Answer C: In regards to an ischemic evaluation given risk factors of hypertension, diabetes and obesity, her age and sex make this diagnosis less likely. FDG-PET imaging can include nuclear stress imaging to evaluation for potential reversible ischemia as a cause of her heart failure as well.

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